

D-004 — HCOS™ Human Load Protection

A Framework for Protecting People from Unnecessary System Pressure

Version 1.0 — Draft

Sophia Care Alliance™
Human-Centered Operating Systems™

Document field	Information
Discipline code	D-004
Document title	HCOS™ Human Load Protection
Version	1.0 — Draft
Document date	July 19, 2026
Document owner	Sophia Care Alliance™
Development status	Foundational draft; substantive content incomplete
Evidence status	Citation and evidence review pending

Development and AI Disclosure

Developed by Sophia Care Alliance™ in collaboration with artificial intelligence using HCOS™ foundations, governance principles, disciplines, standards, and evidence-informed reasoning as guidance.

Artificial intelligence supported document organization, conceptual comparison, language development, and preliminary synthesis. Human authorship, evidence verification, interpretation, professional review, revision, publication, implementation, and accountability remain essential.

This document presents a developing discipline of practice. It does not claim that every HCOS™ concept, framework, method, standard, or instrument described within it has been scientifically validated.

Executive Summary

Every human system creates demands.

People use attention, effort, time, judgment, emotion, relationships, physical energy, and personal resources to perform work, care for others, solve problems, navigate institutions, and sustain organizations.

Pressure is not inherently harmful.

Learning requires effort. Responsibility requires attention. Meaningful work can be demanding. Growth may involve uncertainty, challenge, and discomfort.

The problem begins when system demands become excessive, chronic, poorly distributed, unnecessary, or impossible to recover from.

In unhealthy systems, people frequently become the mechanisms through which unresolved problems are absorbed.

They remember what the workflow does not reliably track. They work beyond scheduled hours. They repair communication gaps. They manage emotional fallout. They compensate for inadequate staffing. They navigate conflicting expectations. They absorb uncertainty. They continue working after recovery has disappeared.

The system may appear to function because people continue holding it together.

This can conceal the true condition of the system.

HCOS™ Human Load Protection is the discipline concerned with recognizing, assessing, reducing, redistributing, and governing the pressure that human beings carry within systems.

Its purpose is not to eliminate work, responsibility, challenge, or professional judgment.

Its purpose is to ensure that human effort remains:

- Necessary
- Proportional
- Supported
- Fairly distributed
- Recoverable
- Meaningful
- Sustainable
- Consistent with human dignity

The discipline asks five central questions:

1. What pressure exists?
2. Who is carrying the pressure?
3. Should the system be carrying more of this burden?
4. What protections are missing?

5. What system redesign would reduce unnecessary human load?

The central proposition of the discipline is:

Healthy systems do not depend on people absorbing unlimited pressure in order to keep the system functioning.

Human Load Protection helps organizations move beyond individual coping responses and examine the structures producing pressure.

It provides a foundation for standards, methods, assessments, documentation practices, governance processes, AI assistants, dashboards, educational resources, and system redesign tools.

1. Overview

HCOS™ Human Load Protection is an applied discipline concerned with how systems create, distribute, intensify, conceal, and reduce pressure on human beings.

It examines the combined demands placed on people through:

- Workload
- Cognitive effort
- Emotional labor
- Administrative burden
- Coordination
- Uncertainty
- Moral responsibility
- Relationship management
- Physical demand
- Technology
- Decision-making
- Time pressure
- Role conflict
- Exposure to distress
- Lack of recovery

Human load is not identical to workload.

Workload generally describes the amount or volume of work assigned to a person or group. Human load is broader. It includes the total pressure a person must absorb to function within a system, including burdens that may not appear on a task list.

For example, a worker may carry a manageable number of assigned tasks while also carrying:

- Constant interruptions
- Conflicting instructions
- Fear of retaliation
- Responsibility without authority
- Unclear priorities

- Emotional exposure
- Inefficient technology
- Repeated follow-up
- Hidden coordination work
- Lack of recovery

The visible workload may appear reasonable while the total human load is unsafe.

Human Load Protection may be applied within:

- Healthcare
- Education
- Government
- Public safety
- Social services
- Technology
- Artificial intelligence
- Manufacturing
- Retail
- Transportation
- Nonprofit organizations
- Community organizations
- Professional services
- Caregiving systems
- Remote and digital work
- Research
- Organizational leadership

Potential users include:

- Workers
- Professionals
- Teams
- Managers
- Executives
- Organizational leaders
- Human-resources professionals
- Safety professionals
- Quality-improvement teams
- Human-factors specialists
- Educators
- Researchers
- Policy designers
- Technology developers
- AI governance bodies
- Community leaders
- Workers' representatives
- Patients, students, clients, and other system participants

HCOS™ Human Load Protection is the discipline concerned with recognizing, assessing, governing, and redesigning the conditions through which systems place pressure on people, so that human effort remains necessary, proportional, supported, recoverable, and sustainable.

2. Why This Discipline Exists

Most organizations measure what they produce.

They may track:

- Productivity
- Revenue
- Cost
- Speed
- Quality
- Completion rates
- Service volume
- Customer satisfaction
- Utilization
- Efficiency
- Compliance
- Output

These measures may be important.

However, systems often measure outcomes without adequately measuring the human effort required to produce them.

A system may meet its performance targets because people:

- Work through breaks
- Remain available after hours
- Develop personal workarounds
- Carry incomplete information
- Perform unpaid coordination
- Reassure distressed people
- Correct technology errors
- Repeat administrative work
- Absorb conflicting expectations
- Continue despite inadequate recovery

When this effort is not measured, it becomes invisible.

When invisible effort becomes normal, the system may begin to depend on it.

This creates a dangerous interpretation:

The organization assumes the system is working because the work is still getting done.

The continued production of outcomes does not necessarily mean that the system is healthy.

It may mean that people have not yet reached the point at which they can no longer compensate.

Many organizations respond to overload by offering individuals:

- Resilience education
- Stress-management programs
- Mindfulness
- Wellness resources
- Time-management training
- Self-care guidance
- Employee assistance programs

These supports may help individuals and may be valuable when offered appropriately.

They become inadequate when they are used as substitutes for correcting system conditions.

A person cannot use mindfulness to create adequate staffing.

A worker cannot self-care their way out of conflicting organizational priorities.

A professional cannot use time management to make an impossible workload possible.

A team cannot become resilient enough to remove chronic policy contradictions.

HCOS™ Human Load Protection exists to move the question from:

Why is this person struggling?

to:

What is this system asking people to carry, and is that demand necessary, supported, and sustainable?

3. The Human Problem

The central human problem addressed by this discipline is the transfer of unresolved system pressure onto people who may lack the authority, resources, or protection required to change the conditions producing it.

Human load may accumulate through several pathways.

Visible Work

This includes tasks formally assigned, recorded, scheduled, or measured.

Examples include:

- Appointments
- Reports

- Calls
- Production targets
- Documentation
- Meetings
- Customer requests
- Teaching responsibilities
- Clinical responsibilities

Hidden Work

This includes effort required to keep the formal process functioning but not recognized within the process.

Examples include:

- Tracking missing information
- Correcting errors
- Reminding other departments
- Repeating requests
- Coordinating between systems
- Reformatting information
- Reassuring distressed people
- Translating unclear instructions
- Creating unofficial workarounds

Cognitive Load

This includes the mental effort required to:

- Remember information
- Shift attention
- Interpret ambiguity
- Prioritize competing demands
- Detect errors
- Make decisions
- Monitor risk
- Navigate multiple technologies
- Maintain situational awareness

Emotional Load

This includes the effort required to:

- Manage personal emotions
- Respond to distress
- De-escalate conflict
- Maintain professionalism
- Conceal frustration
- Support others

- Absorb criticism
- Continue after difficult events

Relational Load

This includes the work required to sustain trust, communication, cooperation, and relationships within fragmented or high-pressure systems.

Moral and Ethical Load

This occurs when people:

- Know what should be done but cannot do it
- Must choose between competing obligations
- Feel responsible for preventable harm
- Work in conditions that conflict with professional values
- Lack the authority to correct a known problem

Administrative Load

This includes:

- Duplicative documentation
- Approval processes
- Reporting requirements
- Scheduling
- Forms
- Inbox management
- Compliance steps
- Repeated data entry
- Navigating policies

Uncertainty Load

This occurs when people must continue acting without:

- Clear expectations
- Reliable information
- Stable policies
- Defined authority
- Timely feedback
- Predictable resources

Physical Load

This includes bodily effort, exposure, environmental strain, shift work, limited rest, and ergonomic demand.

Recovery Debt

Recovery debt develops when pressure is repeatedly carried without sufficient opportunity for physical, cognitive, emotional, or relational restoration.

Over time, these demands may contribute to:

- Exhaustion
- Burnout
- Reduced attention
- Errors
- Withdrawal
- Cynicism
- Conflict
- Turnover
- Reduced creativity
- Loss of meaning
- Moral distress
- Health consequences
- Deterioration of trust
- Unsafe care or service
- Collapse of informal support structures

Individuals are frequently blamed for these outcomes even when the underlying drivers are structural.

In Simple Terms

Imagine a system is a shopping cart with a broken wheel.

The system still needs to move, so a person pushes harder.

At first, pushing harder may work.

Over time, the person becomes tired or injured because the cart was never repaired.

Human Load Protection asks:

- Why is the cart so hard to push?
- Who keeps pushing it?
- Can the wheel be repaired?
- Can the load be reduced?
- Can someone else help?
- Does the cart need to stop until it is safe?

The goal is not to teach people to push broken carts forever.

The goal is to improve the system.

4. Scope of the Discipline

4.1 Primary Areas of Inquiry

HCOS™ Human Load Protection examines:

- Sources of system pressure
- Distribution of work
- Hidden work
- Human capacity
- Cognitive burden
- Emotional labor
- Moral distress
- Administrative burden
- Responsibility and authority
- Workflows
- Technology-mediated work
- Recovery
- Staffing and resources
- Time pressure
- Role clarity
- Communication
- Psychological safety
- Workarounds
- Escalation
- Stopping points
- Human consequences of system design
- Burden transfer during improvement or automation

4.2 Primary Areas of Practice

The discipline supports:

- Human load recognition
- Load mapping
- Pressure-point analysis
- Recovery assessment
- Workflow review
- Responsibility mapping
- System redesign
- Leadership review
- Technology-impact assessment
- AI implementation review
- Workload governance
- Documentation of load-related concerns
- Intervention planning
- Monitoring and evaluation
- Education and professional development

4.3 What the Discipline Does Not Replace

Human Load Protection does not replace:

- Medical or mental-health diagnosis
- Occupational health evaluation
- Emergency response
- Workplace safety law
- Labor law
- Disability accommodation processes
- Professional licensure requirements
- Staffing regulations
- Union representation
- Human-resources procedures
- Clinical assessment
- Legal advice
- Formal risk-management programs

It may help identify when these specialized protections or processes are required.

4.4 Relationship to Burnout

Burnout is one possible outcome associated with prolonged occupational pressure.

Human Load Protection is broader than burnout prevention.

It examines system pressure before, during, and after visible harm develops. It also applies to burdens that may produce error, disengagement, inequity, conflict, moral distress, or reduced system reliability without necessarily meeting a formal definition of burnout.

4.5 Relationship to Workload Management

Workload management focuses primarily on the amount and distribution of work.

Human Load Protection also examines:

- Hidden work
- Emotional demand
- Uncertainty
- Technology friction
- Responsibility without power
- Moral pressure
- Recovery
- System stopping points

4.6 Relationship to Individual Well-Being

Individual well-being matters.

However, Human Load Protection begins with the proposition that human well-being cannot be separated from system design.

The discipline does not treat people as responsible for independently adapting to every condition imposed on them.

5. Foundational Premises

Premise 1: Every System Creates Human Load

Systems require people to use effort, time, attention, judgment, and emotion.

The relevant question is not whether load exists, but whether the load is necessary, proportionate, supported, and recoverable.

Practice implication

Every major system design or organizational change should include a human-load review.

Risk if ignored

Organizations may increase expectations without understanding cumulative demand.

Premise 2: Human Capacity Is Finite

People cannot maintain unlimited attention, emotional regulation, decision-making, or physical effort.

Practice implication

Capacity should be treated as a system-design condition rather than a character trait.

Risk if ignored

Organizations may interpret overextension as commitment and normal capacity as underperformance.

Premise 3: Systems Shape Human Performance

Performance occurs within conditions.

Workload, time, information, technology, resources, authority, leadership, and recovery influence what people can realistically do.

Practice implication

Repeated human difficulty should trigger a system review.

Risk if ignored

Individuals may be retrained or disciplined while the same conditions continue producing the same outcomes.

Premise 4: Humans Must Not Become Permanent Shock Absorbers

People often protect systems by absorbing disruptions, errors, shortages, and uncertainty.

Temporary adaptation may be necessary during an emergency.

It should not become the permanent operating model.

Practice implication

Recurring workarounds should be treated as evidence of system strain.

Risk if ignored

Invisible compensation becomes institutionalized.

Premise 5: Responsibility Must Follow Power

People with authority over policies, staffing, workflow, technology, resources, and priorities carry responsibility for the human consequences of those decisions.

Practice implication

The person experiencing pressure should not automatically be assigned responsibility for correcting it.

Risk if ignored

Organizations may ask people with the least power to solve problems created by those with the most power.

Premise 6: Recovery Is a System Requirement

Recovery supports attention, judgment, emotional regulation, learning, creativity, and sustainable performance.

Practice implication

Recovery should be included in staffing, scheduling, workload, technology, and implementation decisions.

Risk if ignored

People may continue functioning temporarily while their capacity steadily declines.

Premise 7: Pressure Requires Stopping Points

Systems need defined conditions under which work pauses, escalation occurs, demand is redistributed, or the process is redesigned.

Practice implication

People should know when and how they may stop an unsafe or unreasonable process.

Risk if ignored

Pressure may continue simply because no one feels authorized to interrupt it.

Premise 8: Human Dignity Takes Priority Over Operational Convenience

People are not disposable inputs.

Practice implication

Efficiency decisions should be evaluated for their effect on dignity, safety, agency, and sustainable human participation.

Risk if ignored

People may be reduced to capacity, productivity, cost, or compliance metrics.

Premise 9: Improvement Must Not Merely Relocate Load

A change may reduce pressure in one part of a system while transferring it elsewhere.

Practice implication

All redesigns should examine where work and responsibility move.

Risk if ignored

Efficiency gains may conceal new burden placed on people with less visibility or power.

Premise 10: Healthy Systems Learn Before People Break

Organizations should not wait for burnout, turnover, injury, failure, or crisis before responding.

Practice implication

Early warning signals should trigger review and adaptation.

Risk if ignored

Human harm becomes the evidence required to prove that a system was unsafe.

6. Core Principles

Principle 1: Healthy Systems Protect Humans

Statement:

Systems should actively protect people from unnecessary, disproportionate, or unrecoverable pressure.

Meaning:

Protection is a design responsibility, not merely an individual coping strategy.

Why it matters:

Human capacity cannot remain the final resource used to compensate for every failure.

Practice implications:

- Assess pressure during design.
 - Create escalation pathways.
 - Include recovery.
 - Monitor cumulative burden.
 - Respond before serious harm occurs.
-

Principle 2: Pressure Should Be Intentional**Statement:**

Systems should be able to explain why a demand exists.

Meaning:

Not all work is necessary simply because it has historically been performed.

Why it matters:

Outdated, duplicative, or poorly designed requirements consume limited human capacity.

Practice implications:

- Examine the purpose of each major demand.
 - Remove requirements that no longer serve a valid need.
 - Distinguish essential work from inherited process.
 - Reassess policies when conditions change.
-

Principle 3: Pressure Should Be Proportional**Statement:**

The level of human demand should be proportionate to the importance, risk, and available resources associated with the task.

Meaning:

Low-value work should not consume disproportionate human effort.

Why it matters:

Excessive demand reduces capacity for work that matters more.

Practice implications:

- Match effort to risk.
 - Simplify low-value processes.
 - Reduce unnecessary approvals.
 - Avoid excessive documentation.
 - Prioritize clearly.
-

Principle 4: Pressure Should Be Recoverable**Statement:**

People should have meaningful opportunities to recover after sustained or intense demand.

Meaning:

A temporary high-pressure period may be manageable when it is followed by relief.

Why it matters:

Chronic pressure without recovery produces cumulative risk.

Practice implications:

- Build recovery into schedules.
 - Limit repeated surges.
 - Protect time away from work.
 - Avoid normalizing after-hours compensation.
 - Provide decompression after high-impact events.
-

Principle 5: Load Must Be Visible**Statement:**

Systems should identify both visible and hidden human work.

Meaning:

Unmeasured work is still work.

Why it matters:

Decisions based only on formal tasks underestimate the actual effort required.

Practice implications:

- Document workarounds.
- Include coordination and emotional labor.
- Examine administrative friction.
- Ask people what the process actually requires.

- Compare official and real workflows.
-

Principle 6: Load Must Be Fairly Distributed

Statement:

Pressure should not be concentrated on people because they are conscientious, accessible, compassionate, less powerful, or historically expected to compensate.

Meaning:

Systems often direct extra work toward the people most likely to complete it.

Why it matters:

Reliability and care can become reasons that certain individuals are repeatedly overloaded.

Practice implications:

- Monitor distribution.
 - Examine unpaid and informal work.
 - Protect high-reliability contributors.
 - Avoid assigning every unresolved problem to the most responsive person.
-

Principle 7: Responsibility Must Match Authority

Statement:

People should not be held responsible for outcomes they lack the authority, resources, or information to influence.

Meaning:

Responsibility without power creates moral and operational strain.

Why it matters:

People may be blamed for predictable failures created by decisions beyond their control.

Practice implications:

- Map decision authority.
 - Provide resources with responsibility.
 - Create escalation paths.
 - Hold decision-makers accountable for structural conditions.
-

Principle 8: Recovery Is Productive Infrastructure

Statement:

Recovery is necessary for sustained quality, learning, safety, and judgment.

Meaning:

Recovery should not be treated as lost productivity.

Why it matters:

People cannot indefinitely perform complex work without restoration.

Practice implications:

- Protect breaks.
 - Design realistic schedules.
 - Monitor recovery debt.
 - Include recovery in capacity planning.
 - Avoid rewarding chronic overextension.
-

Principle 9: Systems Should Adapt Before Harm Occurs

Statement:

Early warning signs should lead to system response.

Meaning:

Organizations should not require crisis-level evidence before acting.

Why it matters:

Waiting for visible collapse transfers the cost of proof onto people.

Practice implications:

- Use leading indicators.
 - Review recurring workarounds.
-
-

Appendix A — Repository and Publication Information

Repository workflow note: This appendix explains where D-004 belongs within the HCOS™ GitHub architecture. It does not change the discipline's conceptual content.

A.1 Repository Placement

D-004 should be stored within the central **HCOS™ Disciplines** repository:

```
HCOS-Disciplines/  
└── disciplines/  
    └── D-004-Human-Load-Protection/
```

The primary discipline document should be stored at:

```
HCOS-Disciplines/  
└── disciplines/  
    └── D-004-Human-Load-Protection/  
        └── D-004-HCOS-Human-Load-Protection.md
```

A.2 Repository Identification

Repository element	Recommended value
Main repository	HCOS-Disciplines
Discipline folder	disciplines/D-004-Human-Load-Protection/
Primary file	D-004-HCOS-Human-Load-Protection.md
Discipline code	D-004
Official discipline name	HCOS™ Human Load Protection
Version	Version 1.0 — Draft
Document owner	Sophia Care Alliance™
Framework	Human-Centered Operating Systems™
Evidence status	Evidence development and review pending
Professional review status	Pending
Public-use status	Developmental educational and framework material

A.3 Repository Description

Full description:

HCOS™ D-004 Human Load Protection provides a human-centered framework for recognizing, assessing, reducing, redistributing, and governing unnecessary system pressure before people are harmed.

Short GitHub description:

A human-centered systems discipline for identifying pressure, hidden work, recovery gaps, responsibility imbalances, and opportunities to reduce unnecessary human load.

A.4 Initial Upload Structure

Only the primary discipline document is needed to begin:

```
D-004-Human-Load-Protection/  
└── D-004-HCOS-Human-Load-Protection.md
```

Do not create empty folders solely to represent future work.

A.5 Expanded Repository Structure

Add folders as resources are actually developed:

D-004-Human-Load-Protection/

- |
- |—— README.md
- |—— D-004-HCOS-Human-Load-Protection.md
- |
- |—— overview/
 - |—— D-004-Executive-Summary.md
 - |—— D-004-One-Page-Overview.md
 - |—— D-004-ELI5-Guide.md
- |
- |—— framework/
 - |—— Human-Load-Protection-Framework.md
 - |—— Five-Question-Human-Load-Model.md
 - |—— Human-Load-Taxonomy.md
 - |—— Human-Load-Protection-Journey.md
 - |—— diagrams/
- |
- |—— governance/
 - |—— Human-Load-Governance-Framework.md
 - |—— Responsibility-and-Authority-Governance.md
 - |—— Escalation-and-Stopping-Points.md
 - |—— Recovery-Protection-Governance.md
 - |—— Human-Load-Decision-Pathway.md
 - |—— Human-Load-Governance-Decision-Log.md
- |
- |—— standards/
 - |—— Human-Load-Assessment-Standard.md
 - |—— Human-Load-Documentation-Standard.md
 - |—— Human-Load-Review-Standard.md
 - |—— Human-Load-Intervention-Standard.md
 - |—— Human-Recovery-Protection-Standard.md
 - |—— Human-Centered-Workload-Design-Standard.md
- |
- |—— methods/
 - |—— Human-Load-Mapping.md
 - |—— Pressure-Point-Analysis.md
 - |—— Recovery-Gap-Assessment.md
 - |—— Cognitive-Load-Review.md
 - |—— Hidden-Work-Analysis.md
 - |—— Responsibility-and-Power-Mapping.md
 - |—— Human-Centered-System-Redesign.md
- |
- |—— instruments/
 - |—— Human-Load-Protection-Prompt.md

- | | | | | — Human-Load-Check-Guide.md
- | | | | | — Human-Load-Scanner.md
- | | | | | — System-Pressure-Mapping-Tool.md
- | | | | | — Recovery-Gap-Assessment.md
- | | | | | — Human-Load-Dashboard.md
- | | | — instrument-development/
- | |
- | — ai-assistants/
- | | | | | — Human-Load-Protection-Assistant.md
- | | | | | — Human-Load-Check-AI.md
- | | | | | — Human-Load-Review-Prompt.md
- | | | — AI-Safety-and-Human-Oversight.md
- | |
- | — evidence/
- | | | | | — Human-Load-Evidence-Map.md
- | | | | | — annotated-evidence-records/
- | | | | | — Burnout-and-Human-Load-Evidence.md
- | | | | | — Recovery-Evidence.md
- | | | | | — Cognitive-Load-Evidence.md
- | | | | | — Research-Gaps.md
- | | | — Research-Agenda.md
- | |
- | — education/
- | | | | | — Learning-Objectives.md
- | | | | | — Foundational-Learning-Guide.md
- | | | | | — Practitioner-Learning-Guide.md
- | | | | | — Leadership-Learning-Guide.md
- | | | | | — Facilitator-Guide.md
- | | | — case-studies/
- | |
- | — implementation/
- | | | | | — Human-Load-Implementation-Guide.md
- | | | | | — Pilot-Protocol.md
- | | | | | — Intervention-Planning-Template.md
- | | | | | — Evaluation-Framework.md
- | | | — Before-and-After-Comparison-Guide.md
- | |
- | — document-governance/
- | | | | | — Document-Status.md
- | | | | | — Authorship-and-AI-Disclosure.md
- | | | | | — Evidence-Review-Status.md
- | | | | | — Review-and-Approval.md
- | | | — Version-History.md

A.6 Folder Functions

Folder	Function
overview/	Shorter versions of the discipline for different audiences.
framework/	Core models, taxonomy, journey, and diagrams.
governance/	Organizational governance of human load, authority, escalation, stopping points, and recovery.
standards/	Repeatable expectations for responsible practice within the discipline.
methods/	Structured approaches for recognizing, assessing, and reducing human load.
instruments/	Prompts, guides, worksheets, assessments, scanners, and dashboards.
ai-assistants/	AI-supported applications, instructions, limitations, and human-oversight requirements.
evidence/	Evidence maps, annotated records, research gaps, and the research agenda.
education/	Learning resources for introductory, professional, leadership, research, and facilitator audiences.
implementation/	Piloting, intervention planning, application, monitoring, and evaluation resources.
document-governance/	Authorship, review, versioning, evidence status, corrections, and publication records for D-004.

A.7 Governance Distinction

governance/

Governs how organizations recognize, assess, escalate, reduce, and remain accountable for human load.

document-governance/

Governs how the D-004 publication is authored, reviewed, revised, approved, versioned, and published.

Human-load governance governs the practice. Document governance governs the publication.

A.8 README Purpose

When created, README.md should serve as the entry point to D-004. It should explain:

- What Human Load Protection is
- Why the discipline exists
- The five central questions
- Who may use the discipline
- The development and evidence status
- Which resources are available
- Which resources remain planned
- How AI-assisted authorship and human review are governed
- How feedback may be submitted

Resources not yet created should be labeled **Planned resource** and should not be presented as active downloads.

A.9 File-Naming Rules

1. Begin major discipline files with D-004 when appropriate.
2. Use descriptive names and hyphens instead of spaces.
3. Preserve the official discipline name.
4. Record version information inside the document rather than in every filename.
5. Do not use filenames such as final, final-new, or latest.
6. Use Git history and the version record to track revisions.

Examples:

D-004-HCOS-Human-Load-Protection.md

D-004-Executive-Summary.md

D-004-One-Page-Overview.md

Human-Load-Protection-Framework.md

Human-Load-Check-Guide.md

Human-Load-Evidence-Map.md

A.10 Publication Status Labels

Use one of the following labels on each resource:

- Concept
- Working draft
- Version 1.0 — Draft
- Under evidence review
- Under expert review
- Pilot version
- Revised draft
- Approved for publication
- Archived
- Superseded

Initial D-004 resources should normally be labeled **Version 1.0 — Draft** or **Planned resource**.

A.11 Provisional Citation

Until a formal citation record is created, use:

Sophia Care Alliance. (2026). *D-004 — HCOS™ Human Load Protection: A framework for protecting people from unnecessary system pressure* (Version 1.0, Draft). Human-Centered Operating Systems™.

A future CITATION.cff file may provide standardized GitHub citation information.

A.12 Suggested GitHub Topics

human-centered-systems
human-load
human-load-protection
systems-thinking
organizational-design
burnout-prevention
human-factors
workload
recovery
ai-governance
hcos
sophia-care-alliance

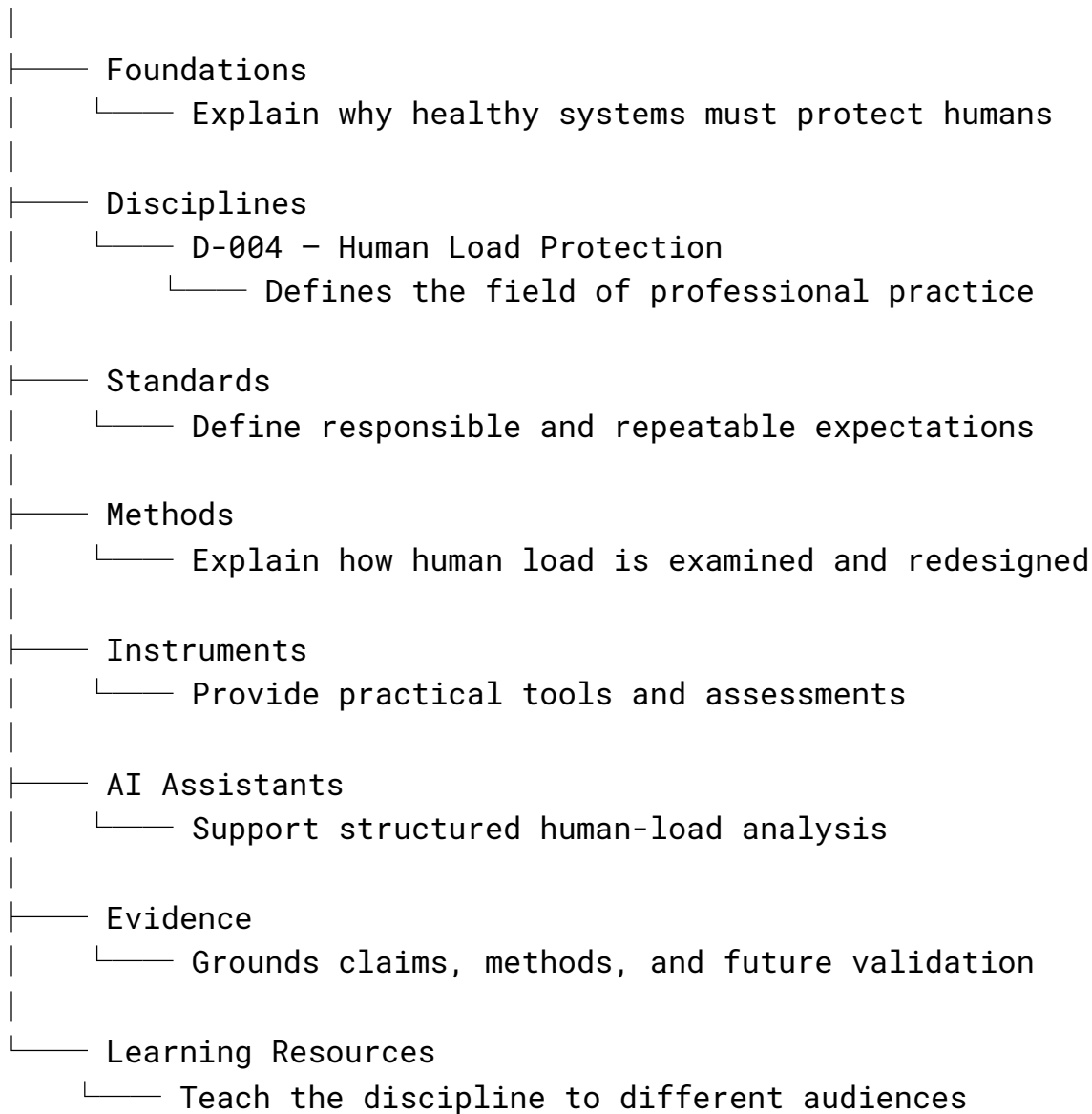
A.13 Recommended Upload Sequence

1. Complete and upload D-004-HCOS-Human-Load-Protection.md.
2. Create README.md.
3. Add the Human Load Check Guide.
4. Add the Human Load Protection Prompt.

5. Create the Five-Question Human Load Model.
6. Establish the evidence folder and evidence map.
7. Develop the initial standards.
8. Add governance and stopping-point guidance.
9. Develop implementation tools.
10. Add document-governance records as formal review begins.

A.14 HCOS™ Repository Hierarchy

HCOS™



D-004 remains a discipline because it defines the field. Standards define responsible practice within the field; methods define structured approaches; instruments apply those methods; and evidence supports the discipline's continued development.

A.15 Current Draft Completion Note

The substantive discipline text currently ends during **Principle 9: Systems Should Adapt Before Harm Occurs**. Complete the remaining discipline sections before applying the publication-readiness and citation-integration workflow below.

Appendix B — Post-Creation Evidence and Citation Review Directive

Repository workflow note: This section governs what must occur after the substantive document draft is completed. It is not part of the discipline's conceptual content.

B.1 Required Review Step

After the initial draft of this document is complete, apply the **HCOS™ Citation and Evidence Integration Prompt (HCOS-CIT-001)** before the document is treated as publication-ready, implementation-ready, or formally issued.

The review must preserve the document's structure, voice, terminology, human-centered purpose, and appropriate evidentiary boundaries.

B.2 Standard Post-Creation Instruction

Apply the HCOS™ Citation and Evidence Integration Prompt to this completed document. Preserve its structure, voice, terminology, and human-centered purpose. Add APA 7 in-text citations and a complete bibliography. Use HCOS™ repository documents for internal framework citations and primary or authoritative external sources for empirical claims. Clearly distinguish established findings, authoritative definitions or guidance, author interpretation, HCOS™ synthesis, proposed application, hypotheses, and future research. Do not invent sources, metadata, quotations, findings, page numbers, URLs, DOIs, or repository records. Do not overstate the evidence. Identify unsupported claims, full-text limitations, and areas requiring human review.

B.3 Required Sequence

1. Complete the substantive document draft.
2. Preserve the uncited draft in the version history.
3. Apply HCOS-CIT-001.
4. Review factual, empirical, historical, legal, regulatory, clinical, technical, and research-based claims.
5. Add accurate APA 7 in-text citations and a corresponding bibliography.
6. Cite internal HCOS™ documents only for HCOS™ definitions, relationships, design history, and proposed framework content.
7. Use primary or authoritative external sources for scientific, clinical, legal, regulatory, statistical, historical, and technical claims.
8. Qualify, narrow, correct, remove, or flag claims that are not fully supported.
9. Complete the citation audit and evidence-integration record.
10. Update the document version, status, and repository history.
11. Obtain identifiable human review before publication, implementation, clinical use, legal reliance, or formal adoption.

B.4 Required Outputs

The evidence-integration review must produce:

1. A revised document with citations integrated
2. A citation audit table
3. An APA 7 bibliography containing every cited source
4. Evidence integration notes
5. A repository update record

B.5 Evidence Status

Completion of citation integration means the document has been **evidence-informed and citation-reviewed**. It does not establish that this discipline, or any related HCOS™ framework, model, method, standard, or instrument, has been independently validated.

Validation may be claimed only when evidence exists for the exact HCOS™ component, intended purpose, population, setting, and use.

B.6 Suggested Version-History Language

Citation and evidence integration completed using HCOS-CIT-001. Claims were reviewed for evidentiary support, citations and bibliography were updated, evidence boundaries were documented, and unresolved claims were identified for human review.

B.7 Required Closing Statement

Citation integration complete. Human review is required before publication, implementation, clinical use, legal reliance, or formal adoption.